

An Autistic Adult in Your Clinic

For family doctors & health professionals — caring for an autistic adult in employment.

The one idea

Autism, through **monotropism**, means an adult's attention locks into a few deep, intense "tunnels." In the right environment this gives real strengths — sustained concentration, reliability, pattern-detection, depth and honesty. In the wrong one (open-plan offices, constant interruption, unclear expectations, office politics) it drives chronic depletion, masking, and **autistic burnout**. Many working autistic adults — especially women — were diagnosed only in adulthood. A 15-minute appointment rarely shows the true cost.

What you may see — and what's really going on

You may see...	What's often happening
Exhausted, can't recover after work, losing skills	Autistic burnout — distinct from occupational burnout and from depression
Anxiety, low mood, sleep or GI problems	Common co-occurrences and overload signals
"Fine" in clinic, collapses for days after	Heavy masking ; the real cost is paid out of sight
Doesn't report pain, symptoms, or side-effects until severe	Interoception differences — internal signals may not register when absorbed
Wants email over calls; needs things in writing	Single-channel, literal communication reduces load

Try this

- **Offer the AASPIRE AHAT** pre-visit and **written-first** options; allow a supporter.
- **Recognise autistic burnout** — chronic exhaustion, loss of function, reduced tolerance to stimulus — and don't confuse it with depression or laziness.
- **Ask about workplace load and recovery**: open-plan, meetings, interruptions, masking.
- **Start medications low and titrate slowly** — autistic adults often report atypical sensory/emotional responses.
- **Signpost workplace accommodations** (Equality Act / ADA) and suggest the person is the expert on what helps.

Avoid this

- Treating burnout as depression and pushing the patient to “try harder” or mask more — this deepens it and raises suicide risk.
- Letting a calm, masked 15 minutes override self-report and history.
- Assuming “not complaining” means “not suffering.”

When to get more support

Assess for **suicidality** — burnout is linked to it. Address co-occurring **ADHD (AuDHD), anxiety, depression, sleep and GI conditions**. Where burnout is severe, validate **time off or reduced expectations** as a recovery step, not weakness. Document effective accommodations so they are automatic next time.

If the patient is a woman

Women are over-represented among the “lost generation” of late-diagnosed adults and mask most heavily. Take a lifelong pattern of social/sensory difference and private exhaustion seriously, even with a “successful” exterior.

About this guide

*AI-assisted, derived from this project's research drafts — the **Monotropism** brief and the **Lifespan Practitioner & Health-Care guide** (Parts 3.4 & 4). Uses identity-first language. **Informational — not medical advice or a diagnostic tool**. Fit it to the individual.*

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2023); Dwyer (2025); Raymaker et al. (2020); Mantzalas et al. (2022, burnout protective factors); Pearson & Rose (2021, masking); Knight (2021); Kelly Mahler (2024, interoception); AASPIRE AHAT. Full citations are in the source drafts.